

STRATEGY BOARD ASSESSMENT

Altered States and the Executive Healthspan Position

Prepared for Vladislav Andreev, O!Sapiens

Strategic decision document. August 2026.

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Option F. Sequenced transition, operated as Option B, with Option E as the standing architecture.

Psychedelics should not enter Vlad's LinkedIn positioning now. They should enter indirectly, later, and only as a small subordinate clause inside a larger argument about state regulation and behaviour change. The recommendation is Option F, a sequenced 90 day test, executed in practice as Option B, the soft bridge, with Option E, a separate property for depth work, as the permanent structural home for anything that goes further. Options C and D are rejected. A controlled merge and a full repositioning both fail on the same point: they trade a category Vlad can own alone for a category that is already occupied, already discredited in the eyes of corporate buyers, and unsupported by evidence for the specific claim that would need to be true, which is that altered states improve leadership capacity.

The most important finding in this assessment is not about psychedelics. It is that the people advising Vlad to go public are right that something is missing from the current content, and wrong about what it is. Sapiens OS already contains four pillars that are explicitly existential: Agency, Meaning, Rest, and Curiosity. None of them have been written. Two pillars have been published, Sleep and Nutrition, and both are physiological. The perceived shallowness of the Executive Healthspan position is a content execution gap, not a positioning gap. It can be closed at zero reputational cost by writing the four pillars that already exist in the framework. Adding psychedelics to solve it would be repositioning the brand to fix a publishing backlog.

THE ONE SENTENCE ANSWER

Write Agency, Meaning, Rest, and Curiosity first. If depth is still missing after those four pillars are published, the problem was never the absence of psychedelics.

2.1 Is the current position too narrow, too sanitised, or correctly disciplined?

Correctly disciplined, and under-executed. These are different problems with different solutions and they are currently being confused.

The Executive Healthspan position passes every test that matters. It is specific, it is defensible, it names a mechanism, and no significant competitor holds the same ground with the same combination of scientific fluency and commercial credibility. The problem is that only two of ten pillars have been published, and both of them are the physiological ones. A reader encountering the brand today sees sleep, nutrition, HRV, and VO2 max. That reader would reasonably conclude the brand is about recovery metrics. It is not. It is about the biological conditions for sustained agency, and four of the ten pillars say so directly.

This is why the advice to add psychedelics feels intuitively right to people who know Vlad's actual range. They are detecting a gap between the man and the output. The correct response is to close the gap with the material already in the framework, not to import a new and heavily loaded category.

2.2 Is the psychedelic layer a missing truth, a differentiator, a distraction, or a separate business line?

It is a separate business line that is currently being mistaken for a differentiator.

It is a genuine part of Vlad's expertise and lived experience, which is why it reads as a missing truth. But truth and positioning are different questions. The relevant test is not whether the knowledge is real. It is whether making it public on LinkedIn strengthens the commercial position. On the evidence, it does not:

- The specific claim that would need to be true, that altered states improve leadership or executive capacity, has no controlled evidence supporting it. Not weak evidence. None.
- The thing executives actually ask about, microdosing, has been repeatedly tested and does not beat placebo. A 2026 preregistered meta-analysis of 14 studies found no overall cognitive benefit and a selective reduction in cognitive control.
- The category is already occupied by well-funded incumbents with a different risk posture, and Bloomberg has already characterised it as an eccentric offshoot of an unregulated coaching field.
- The corporate buyer risk is not theoretical. A named tech CEO was removed by his board after microdosing before an investor meeting. That is the exact scenario a client would be silently modelling.

2.3 The main strategic danger

CATEGORY CAPTURE

Psychedelics is an absorbing label, not an additive one. Once a professional is publicly associated with it, every other thing they do is read through it. Vlad would not become an Executive Healthspan coach who also understands altered states. He would become the psychedelic guy who also talks about sleep. The label does not sit alongside the position. It replaces it.

The asymmetry is severe. Entering the category takes one post. Exiting it takes years, and the archive never fully clears.

2.4 The main strategic opportunity

BE THE CALM VOICE THAT SAYS THE UNPOPULAR THING

There is a genuine and currently unoccupied position available: the evidence-literate executive advisor who can explain what the research actually shows, including the parts the enthusiasts skip. Almost nobody with commercial credibility is telling founders that microdosing does not beat placebo, that a third of people reporting extended difficulties had them persist beyond a year, or that the strong clinical signal is for treatment-resistant depression under supervision and says nothing about performance in healthy high functioning adults.

This position is safe, differentiating, and available. It requires zero endorsement, zero facilitation, and zero personal disclosure. It also happens to be the position that most protects the people Vlad cares about.

All six options assessed against the twelve decision criteria. Scoring is directional judgement, not measurement.

OPTION	UPSIDE	DOWNSIDE	VERDICT
A. Do not merge	Zero added risk. Position stays clean and precise.	Leaves the depth gap unaddressed. Wastes real expertise. Feels dishonest to Vlad over time.	Insufficient alone
B. Soft bridge	Adds depth through nervous system, agency, meaning, identity, and integration language without naming substances. Low risk. Immediately actionable.	Some in the audience will read it as evasive. Does not satisfy advisors pushing for openness.	RECOMMENDED
C. Controlled merge	Differentiation. Attracts a self-selecting deeper audience.	Category capture. Corporate buyer loss. Platform exposure. Competes against incumbents in a discredited field. No evidence base for the leadership claim.	REJECT
D. Full repositioning	Clarity of a single identity. Strong resonance with a niche.	Abandons a defensible category for an occupied one. Destroys premium corporate access. Irreversible.	REJECT
E. Two brands	Preserves both truths. Depth work has a legitimate home. Already partly built via the existing depth property.	Operational load. Discovery leakage between lanes is real but manageable.	RECOMMENDED as architecture
F. Sequenced test	Decision made on data rather than opinion. Reversible. Protects downside.	Delays resolution by 90 days. Requires discipline to actually stop if signals are negative.	RECOMMENDED as sequence

3.1 Audience effects by option

OPTION	ATTRACTS	REPELS	COMMERCIAL EFFECT
A	Executives seeking performance repair	Nobody meaningful	Neutral. Slow growth.
B	Founders in existential transition. Senior leaders questioning the cost of their role.	Nobody meaningful. Enthusiasts find it too restrained.	Positive. Deepens inbound quality without changing buyer profile.
C	Psychonauts. Curious coaches. A small number of high agency founders.	Corporate buyers. Investors. Risk-averse boards. Conservative C-suite.	Mixed to negative. Volume up, qualified volume down.
D	The psychedelic-interested segment exclusively.	Almost the entire current executive audience.	Negative for premium coaching. Possibly positive for a different business.

OPTION	ATTRACTS	REPELS	COMMERCIAL EFFECT
E	Both audiences, in their correct venues.	Only people demanding a single unified public identity.	Positive. Protects the premium lane.
F	Same as B during the test window.	Same as B.	Neutral during test. Optionality preserved.

3.2 Risk profile by option

OPTION	REPUTATIONAL	PLATFORM	SCIENTIFIC CREDIBILITY	O!SAPIENS FIT
A	None	None	None	Perfect
B	Very low	Very low	None if evidence graded	Strong. Uses existing pillars.
C	High	Moderate	High. The leadership claim is unevidenced.	Weak. Dilutes the biological thesis.
D	Severe	Moderate to high	Severe	Breaks it
E	Low	Low	Low	Protects it
F	Very low	Very low	Low	Strong

**Personal brand as umbrella. O!Sapiens as the clean commercial layer.
Depth work in a separate property.**

Vlad Andreev the person is the umbrella and the only entity permitted to hold both lanes. O!Sapiens is a clean commercial brand that never touches substances in any public communication. The depth property carries the altered states, integration, and consciousness work with its own audience, its own tone, and its own boundaries. The two lanes are connected through Vlad and nowhere else.

4.1 How each element relates

ELEMENT	ROLE	PUBLIC ON LINKEDIN?
Vlad Andreev	The umbrella and the trust vehicle. Holds both lanes. Never denies the depth work, never leads with it on LinkedIn.	Yes, as the whole person
O!Sapiens	Master brand and commercial layer. Executive Healthspan. Never associated with substances publicly.	Yes, primary
Sapiens OS	The ten pillar methodology. Owns the framework IP. Contains the existential pillars that carry the depth.	Yes, primary
SapiensOS HealthSpan	Future product layer. Regulated environment. Must stay maximally clean for medical device and app store reasons.	Yes, later
Executive Healthspan	The category label. Stays as the primary frame.	Yes, primary
Altered states	A subordinate concept inside state regulation. Referenced conceptually. Never as an offer.	Indirect only
Psychedelics	Named only when discussing published research or public policy, never as personal practice or service.	Rare and evidence-only
Integration	Usable as a general behaviour change term. Not usable as a psychedelic service term on LinkedIn.	Yes, with care
Coaching	The commercial service. 1:1 executive work under O!Sapiens.	Yes
Education	The public good layer. Harm reduction and evidence literacy live in the depth property.	Depth property

4.2 The firewall

Three rules keep the architecture intact:

1. O!Sapiens branded assets, the app, the diagnostic, and any corporate or investor material never reference substances in any form. This is not a tone preference. Once the app enters the regulated EU environment, substance-adjacent brand association becomes a material commercial risk.

2. The depth property does not link to O!Sapiens and O!Sapiens does not link to the depth property. Vlad's personal profile may acknowledge both if asked directly. Neither drives traffic to the other.
3. No client crosses lanes without an explicit, separate conversation. Someone who arrived through Executive Healthspan does not get offered depth work.

CANDIDATE	CLARITY	EXEC RELEVANCE	SCI CREDIBILITY	REPUTATIONAL SAFETY	VERDICT
Executive Healthspan	High	High	High	High	PRIMARY . Keep.
Executive Biology	High	High	High	High	Secondary explanatory phrase
Biological Performance	High	Medium	High	High	Usable, less ownable
State-Aware Leadership	Medium	High	Medium	High	Strong bridge phrase
Executive Self-Regulation	High	Medium	High	High	Useful sub-frame
Integration-Based Performance	Low	Low	Low	Medium	Reject. Reads as psychedelic code.
The Biology of Self-Agency	Medium	Medium	Medium	High	Good essay title, weak category
Consciousness and Capacity	Low	Low	Low	Low	Reject
Inner Operating Systems	Low	Low	Low	Medium	Reject. Generic and unprovable.
Altered States for Leaders	High	Low	Very low	Very low	Reject outright

5.1 Recommendation

- Primary category: Executive Healthspan. It is already established, already differentiated, and already understood by the buyer. Changing it now would forfeit accumulated recognition to solve a problem that is not a category problem.
- Secondary explanatory phrase: Performance is biological first. Keep it. It is the strongest asset in the messaging system and it survives the addition of depth material without modification.
- Bridge phrase for the soft merge: state-aware leadership. It carries the altered states concept without the substance association, and it is technically accurate. Attention, judgement, and reactivity are all state dependent.

5.2 Categories to avoid permanently

- Altered States for Leaders. It is a reputational trap with no evidence base. It also invites the specific question Vlad cannot answer commercially, which is what he provides.
- Psychedelic executive coach. Occupied, discredited, and legally fraught in Spain where supply is criminal under Article 368 even where personal use is tolerated.
- Anything using consciousness as the category noun. It reads as spiritual to the corporate buyer regardless of how it is framed.

Version 1. Conservative

CONSERVATIVE

Vlad Andreev works with founders and senior leaders whose performance is being quietly limited by biology they are not tracking. The problem is rarely motivation or discipline. It is that sleep architecture, stress load, metabolic stability, and recovery capacity have degraded to the point where the standard executive toolkit stops returning results. He works through the Sapiens OS framework, ten biological and behavioural systems that set the ceiling on attention, judgement, and emotional regulation. Executive Healthspan is the capacity to keep leading, choosing, and recovering over decades rather than quarters. He is a coach and educator. He does not diagnose, treat, or provide therapy, and he does not work with substances of any kind. This matters to founders because the cost of a degraded decision compounds faster than the cost of a degraded quarter.

Version 2. Balanced

BALANCED. RECOMMENDED.

Vlad Andreev works with founders and senior leaders who are still delivering but paying more for it every year. His starting position is that performance is biological before it is psychological. Attention, judgement, reactivity, and the capacity to change behaviour are all state dependent, and the state is set by systems most leaders have never examined: sleep architecture, stress physiology, metabolic stability, light exposure, recovery, and the degree of genuine agency a person holds over their own week.

The Sapiens OS framework covers ten of these systems. Four of them are physiological. Four are existential, covering agency, meaning, rest, and curiosity. Insight alone does not change behaviour. Biology sets the capacity, and integration is what turns any insight, from any source, into something that survives contact with Monday.

He is a coach and educator, not a doctor, therapist, or facilitator. He does not diagnose, treat, prescribe, or work with substances. This matters because most leadership development assumes a stable biological baseline, and for the people who need it most, that assumption is false.

Version 3. Bold, and still credible

BOLD

Vlad Andreev works with founders and senior leaders at the point where high performance has quietly become a debt. His position is that performance is biological before it is psychological, and that most executive development operates one level too high to be useful.

He works across ten systems that determine capacity. Some are physical. Some are not. Agency, meaning, rest, and curiosity are not soft topics. They are measurable inputs to how a nervous system regulates, and their absence shows up in cortisol, sleep architecture, and the quality of decisions made at four in the afternoon.

He also thinks the field around altered states deserves a more honest conversation than it currently gets from either its promoters or its critics. The clinical evidence for supervised psilocybin in treatment-resistant depression is now genuinely strong. The evidence that low doses improve cognition or creativity in healthy adults is not. Both statements are true at once, and a senior leader deserves to hear both from someone with no commercial interest in either.

He is a coach and educator. He does not diagnose, treat, prescribe, facilitate, or supply anything. This matters because the people making the most consequential decisions are the least likely to have anyone in their life willing to tell them the unglamorous version.

7.1 Fifteen headline options

Executive Healthspan Coach for Founders and Senior Leaders Performance is biological first Sleep, stress, focus, decision clarity O!Sapiens	Low
I help founders and senior leaders find the biology quietly limiting their performance Sapiens OS O!Sapiens	Low
Executive Healthspan Performance is biological first Neuroscience-based protocols for sleep, stress load, recovery and decision clarity	Low
Founder, O!Sapiens Executive Healthspan Coach The ten systems that set your ceiling on attention, judgement and recovery	Low
Executive Healthspan Coach Biology sets the capacity. Behaviour sets the outcome. Sapiens OS for founders and senior leaders	Low
Executive Healthspan Coach Performance is biological first Also writing about agency, meaning and why insight rarely survives Monday	Low
Executive Healthspan Coach for founders I work on the biological and existential systems behind sustained leadership capacity O!Sapiens	Low
Executive Healthspan State-aware leadership Attention, judgement and reactivity are state dependent. The state is trainable.	Low
Executive Healthspan Coach Performance is biological first Writing about nervous system states, agency and behaviour that lasts	Medium
Executive Healthspan Coach Biology, behaviour and the states leaders operate from Sapiens OS O!Sapiens	Medium
Founder, O!Sapiens Executive Healthspan Insight is cheap. Integration is the hard part. Ten systems that make change stick.	Medium
Executive Healthspan Coach I work on capacity, not motivation Biology, altered states of arousal, and behaviour change for senior leaders	Medium
Executive Healthspan Coach Evidence-first on performance biology, including an honest read on what the psychedelic research does and does not show	High
Executive Healthspan Coach Founders, biology and the uncomfortable evidence Including what the altered states literature actually says	High
Executive Healthspan Coach O!Sapiens Also one of the few executive voices willing to read the psychedelic research critically	High

7.2 Fifteen tagline options

Performance is biological first.	Low
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Biology sets the capacity.	Low
Sleep first, then philosophy.	Low
The system matters more than the insight.	Low
Capacity, not motivation.	Low
Behaviour change is a skill, not a willpower problem.	Low
Healthspan, not just lifespan.	Low
Your ceiling is physiological.	Low
Attention is a state, not a trait.	Low
Insight is cheap. Integration is the work.	Medium
Every decision is made from a state.	Medium
State-aware leadership.	Medium
Regulate the system, then question the strategy.	Medium
Insight without integration fades.	Medium
The evidence, including the parts nobody quotes.	High

7.3 Recommended

TOP THREE HEADLINES

1. Executive Healthspan Coach for Founders and Senior Leaders | Performance is biological first | Sleep, stress, focus, decision clarity | O!Sapiens
2. Executive Healthspan Coach | Performance is biological first | Also writing about agency, meaning and why insight rarely survives Monday
3. Executive Healthspan | State-aware leadership | Attention, judgement and reactivity are state dependent. The state is trainable.

TOP THREE TAGLINES

1. Performance is biological first.
2. Insight is cheap. Integration is the work.
3. Capacity, not motivation.

Note on the second recommended headline. It signals depth without naming anything. The phrase about insight not surviving Monday does most of the work, because anyone who has had a

significant insight in any context, therapeutic, contemplative, or otherwise, recognises the problem immediately. That is a self-selecting signal that costs nothing.

Version 1. Conservative bridge

CONSERVATIVE BRIDGE

At some point high performance stops feeling like an achievement and starts feeling like a debt.

You are still delivering. The output still lands. But it costs more than it used to. Sleep is lighter. Decisions feel heavier. You are wired at night and flat in the morning, and the thing you do not say out loud is that you are not sure how long this holds.

My starting position is that this is biological before it is psychological. Not because mindset does not matter, but because mindset work assumes a stable baseline, and by the time most leaders reach me the baseline has already moved.

I work through Sapiens OS, a framework covering ten systems that set the ceiling on attention, judgement, and emotional regulation. Six are physical: sleep, nutrition, movement, stress load, light exposure, and rest. Four are not: agency, meaning, social connection, and curiosity. Both halves are measurable. Both show up in physiology.

Background: twenty years in commercial roles across Europe and the Middle East. Negotiation, brand strategy, P&L. Education at Berkeley, Yale, and IMD. Certified coach. I came to this from operating, not from wellness.

I am a coach and educator. I do not diagnose, treat, or provide therapy, and I will say so plainly if what you need is clinical care.

If any of this is recognisable, the first step is a conversation with no agenda beyond understanding what is actually happening.

Version 2. Controlled merge

CONTROLLED MERGE. RECOMMENDED FOR THE 90 DAY TEST.

At some point high performance stops feeling like an achievement and starts feeling like a debt.

You are still delivering. But it costs more than it used to. Sleep is lighter. Decisions feel heavier. And somewhere underneath the operational questions there is a quieter one about whether the thing you are building is still the thing you want.

Both of those are my work.

My starting position is that performance is biological before it is psychological. Attention, judgement, reactivity, and the capacity to change behaviour are all state dependent. The state is set by systems most leaders have never examined.

Sapiens OS covers ten of them. Six are physical: sleep, nutrition, movement, stress load, light exposure, rest. Four are not: agency, meaning, social connection, curiosity. The second group is not soft. The degree of real control a person holds over their own week is a measurable input to their stress physiology. So is whether the work still means anything to them.

One more thing I have learned, mostly the slow way. Insight is cheap. People have significant realisations regularly, in therapy, in crisis, in stillness, on long walks, and in every other context humans have found. Almost none of it changes behaviour on its own. What changes behaviour is a nervous system with enough capacity to hold a new pattern, and a structure that carries the insight into an ordinary Tuesday. That second part is most of the work and it is the part almost nobody sells.

Background: twenty years in commercial roles across Europe and the Middle East. Berkeley, Yale, IMD. Certified coach.

I am a coach and educator. Not a doctor, therapist, or facilitator. I do not diagnose, treat, prescribe, or work with substances of any kind. If what you need is clinical care I will say so.

If this is recognisable, the first step is a conversation with no agenda beyond understanding what is going on.

Version 3. Bold, still credible

BOLD

Most executive development operates one level too high to be useful.

It addresses strategy, habits, and mindset, and assumes the person receiving it has a stable biological baseline. For the leaders who need it most, that assumption is false. You cannot out-discipline a dysregulated nervous system, and no amount of clarity survives four hours of cortisol.

So I start lower. Performance is biological first.

Sapiens OS covers ten systems that set capacity. Six physical, four existential. Agency, meaning, connection, and curiosity belong in a performance framework because they are measurable inputs to physiology, not because they are pleasant to talk about.

I also think the conversation around altered states and leadership deserves better than it currently gets. The clinical evidence for supervised psilocybin in treatment-resistant depression is now genuinely strong, with three positive phase three trials. The evidence that low doses improve cognition or creativity in healthy high performers is not: a 2026 meta-analysis of fourteen studies found no overall benefit and a small reduction in cognitive control. Both of those are true simultaneously, and a founder deciding what to do with their own mind deserves to hear both from someone selling neither.

What I actually work on is the unglamorous part. Insight is cheap. Integration, meaning the structure that carries a realisation into behaviour that survives a hard week, is the entire difference between a meaningful experience and a changed life.

Background: twenty years commercial, across Europe and the Middle East. Berkeley, Yale, IMD. Certified coach.

I am a coach and educator. I do not diagnose, treat, prescribe, facilitate, supply, or advise on the use of anything. If what you need is clinical care I will say so directly.

If this is recognisable, start with a conversation.

9.1 Content ratio

TYPE	SHARE	PURPOSE
Executive Healthspan and Sapiens OS pillars	50%	Core position. Non-negotiable majority. Includes the four unwritten existential pillars.
Recognition and mechanism posts	20%	The this is exactly my situation response. Drives DMs.
Evidence and research posts	15%	Authority. Where graded evidence on states and behaviour change lives.
Personal data and story posts	10%	Trust through demonstrated practice. HRV, VO2, tracked biology.
Boundary and integration posts	5%	The soft bridge. Where depth signals live. Never more than this.

THE HARD RULE

The bridge content never exceeds one post in twenty. If altered states material becomes more than five percent of output, the category capture process has already started, whether or not it feels that way from the inside.

9.2 Sequence

- Days 1 to 30: publish Movement, Stress Regulation, Light and Darkness. Zero bridge content. This closes the physiological half of the framework.
- Days 31 to 60: publish Agency, Meaning, Rest, Curiosity. These four are the real test. If the depth gap closes here, the strategic question is answered without any risk taken.
- Days 61 to 90: publish Social Connection. Introduce a maximum of three bridge posts. Measure response quality carefully against the signal criteria in section 16.

9.3 How to avoid attracting the wrong audience

4. Never use the vocabulary of the scene. Ceremony, medicine, journey, facilitator, sitter, integration circle, and set and setting are search terms that summon a specific audience. Their absence is a filter.
5. Always anchor to executive consequence. If a post about states does not end at decision quality, team effect, or capacity, it is not an Executive Healthspan post.
6. Never write in the first person about personal use. The moment Vlad becomes the subject, the topic becomes the practice rather than the evidence.
7. Lead with the limitation. Posts that open with what the evidence does not support repel enthusiasts and attract skeptical senior readers. This is the correct trade.

8. Do not engage the enthusiast comment thread. Respond to executive-framed comments in depth. Respond to scene-framed comments once, briefly, and neutrally.

9.4 Thirty six post ideas

Recognition posts

- The moment you notice you are rehearsing conversations at 2am and calling it preparation.
- The specific tiredness that sleep does not fix, and why it is not a sleep problem.
- Why the same decision feels obvious on Tuesday morning and impossible on Thursday evening.
- You have not lost your edge. Your recovery window has narrowed and nobody told you.

Mechanism posts

- Why the part of your brain that would notice you are impaired is the first part impairment takes offline.
- Cortisol does not clear on a schedule. What that means for anyone with back to back weeks.
- Glucose stability and the four in the afternoon decision. The metabolic cost of context switching.
- Why your team reads your nervous system state before they read your words.
- Threat detection and the narrowing of cognitive options. What actually happens in a room where someone dominates.

Evidence posts

- What a 2026 meta-analysis of fourteen microdosing studies actually found, and why nobody quotes it.
- Three positive phase three trials for supervised psilocybin in treatment-resistant depression. What that does and does not tell a healthy founder.
- The 175 causal connections in the sleep systems model, and why sleep is a readout rather than a habit.
- Biomarkers that move before burnout is diagnosed. Your last blood panel may have already said something.
- Why observational psychedelic studies show benefit and controlled ones do not. The expectancy problem, explained.
- How to read a study when the field has an incentive to overstate it. A short guide for skeptical operators.

Personal story posts

- My VO2 max fell ten percent while I was performing normally. The data caught what I did not.
- A mid-week HRV drop, traced backwards. Stress load, inflammation, and eleven days without training.
- The year I optimised everything and got worse. What I had wrong about the order of operations.
- An autoimmune diagnosis is a very direct way to learn that biology is not negotiable.

Boundary-setting posts

- What I do not do, and why the list matters more than the list of what I do.

- The difference between coaching and therapy, and why blurring it hurts the person paying.
- Why I will not answer certain questions in DMs, whoever is asking.
- On being asked for things I am not qualified to give.

Integration posts

- Insight is cheap. Almost every leader I work with already knows what is wrong.
- Why realisations do not survive contact with a hard week, and what would have to be true for them to.
- The gap between knowing and doing is not motivation. It is capacity, and capacity is physiological.
- Behaviour change is a skill. Nobody taught it to you, and your competence elsewhere is why you assume you have it.
- What actually has to be in place on the Monday after any significant realisation.

Executive Healthspan posts

- Healthspan is not about living longer. It is about how long you can keep choosing.
- The compounding cost of a degraded decision versus a degraded quarter.
- What your board is modelling about your capacity that nobody will say to you.
- Performance without collapse. Why the two are usually treated as the same trade and are not.

Sapiens OS posts

- Pillar 7, Agency. Why the amount of real control you hold over your own week shows up in your cortisol.
- Pillar 8, Meaning. Not motivation. A regulatory input, and what happens physiologically when it goes.
- Pillar 9, Rest. The distinction between time off and parasympathetic recovery, and why most leaders get none of the second.
- Pillar 10, Curiosity. Cognitive flexibility as a biological capacity that stress removes first.

Conversation-starting posts

- Poll: when in the day do you make your worst decisions? Four options, all biologically distinct.
- What is the thing you know you should change and have not? No advice in the comments, just naming it.
- Ask: what did you stop doing when things got busy, and what did it cost you?

10.1 Zones

GREEN ZONE. Publishable without restriction.

Nervous system states and their effect on judgement. Stress physiology. Sleep architecture. Recovery. Behaviour change science. Agency and meaning as physiological inputs. Integration as a general behaviour change concept. Breathwork and meditation as state regulation practices, described accurately and without overclaiming. Published research findings, correctly graded. Public policy and regulatory developments. Harm reduction principles stated in general terms.

YELLOW ZONE. Only with explicit evidence grading and executive framing.

Named clinical trial results for psilocybin, MDMA, ketamine, or LSD. The state of regulatory approval in the EU, Spain, or the US. The gap between clinical evidence and performance claims. Critical analysis of the microdosing literature. The existence of the altered states field as a cultural phenomenon in founder circles. Extended difficulties and adverse event data. Each of these requires a stated evidence grade and must land on an executive consequence.

RED ZONE. Never publish, in any framing, on any channel connected to O!Sapiens.

Personal use, past or present. Any description of a personal experience with a substance. Client experiences with substances, even anonymised. Dosage, sourcing, preparation, or protocol information. Retreat, clinic, facilitator, or provider recommendations. Legal workarounds or jurisdictional advice. Any suggestion that a substance would help a named person or condition. Any framing of substances as improving leadership, creativity, or performance. Photography, imagery, or symbolism associated with the scene.

10.2 Vocabulary

USE	AVOID
nervous system state	altered state (in headlines)
state dependent	expanded consciousness
published trial evidence	the medicine
supervised clinical setting	ceremony, ritual, sitter
behaviour change structure	integration circle
the research suggests	proven, cure, breakthrough
extended difficulties	bad trip
I am not qualified to advise on that	in my experience with

USE	AVOID
regulatory status	legal grey area
capacity, regulation, recovery	unlock, journey, awakening

10.3 Claims

- Allowed: accurate description of what a published study found, with its population, design, and limitations. Statements about regulatory status sourced to a regulator. Statements that something is not supported by evidence.
- Allowed: general statements that insight requires structure to become behaviour. This is well supported and does not reference substances.
- Forbidden: any claim of therapeutic benefit for a named person or condition. Any comparative claim against conventional treatment. Any statement implying a substance is safe, appropriate, or advisable for anyone. Any implication that Vlad's coaching addresses a diagnosable condition.

10.4 Personal and client story boundaries

- Personal: Vlad may reference having studied the field, having a professional interest in it, and having formed views on the evidence. He may not describe personal use. The distinction is between expertise and practice.
- Client: no client story involving substances is publishable in any form. Anonymisation does not solve this. It creates the impression that substance-involved clients are part of the O!Sapiens practice, which is the exact association the architecture exists to prevent.

10.5 Standing disclaimer language

USE VERBATIM WHERE RELEVANT

This is education, not medical advice. I am a coach, not a doctor, therapist, or facilitator. I do not diagnose, treat, prescribe, or work with substances. Nothing here is a recommendation to use anything. Laws differ by jurisdiction and I am not qualified to advise on them. If you are dealing with something clinical, please work with a licensed professional.

10.6 Response templates

ASKED FOR SCIENTIFIC SOURCES

Happy to point you at the primary literature. For the clinical picture, the Compass phase three publications and the Hinkle et al. adverse event meta-analysis in JAMA Psychiatry are the right starting points. For microdosing specifically, the 2026 Neuroscience and Biobehavioral Reviews meta-analysis is the most rigorous synthesis I know of. I would read the limitations sections closely in all of them.

ASKED FOR A REFERRAL TO A PROVIDER

I do not make referrals in that area and I am not in a position to vouch for anyone. If this is about something clinical, a licensed psychiatrist or psychologist is the right first conversation.

ASKED ABOUT DOSAGE

That is outside what I do and outside what I am qualified to comment on. I will not answer dosage questions from anyone.

ASKED ABOUT CEREMONIES OR RETREATS

Not my area, and not something I have any involvement in. I would not be able to give you a useful or responsible answer.

ASKED ABOUT LEGALITY

Legal status varies significantly by country and changes. I am not a lawyer and would not want you relying on anything I said about it. Worth getting proper advice for your own jurisdiction.

ASKED ABOUT MEDICAL USE

The clinical research is moving fast, and for treatment-resistant depression specifically the phase three data is genuinely notable. But that is a supervised clinical context with a specific diagnosed population. It is a different question from anything I work on, and it is a conversation for a psychiatrist.

ASKED ABOUT TRAUMA TREATMENT

Trauma is clinical territory and I do not work there. That needs a licensed trauma specialist. I would be doing you a disservice if I engaged with it as a coaching question.

ASKED ABOUT MICRODOSING

The honest answer is that the controlled evidence is not encouraging. A 2026 meta-analysis of fourteen studies found no overall cognitive benefit, and one domain, cognitive control, was slightly worse. The observational studies look better, which is usually a sign that expectancy is doing the work. I would not build anything on it.

ASKED ABOUT PERSONAL USE

I keep my own practices out of my professional work, mostly because what I do or do not do is not a useful input to your decision. Happy to talk about what the evidence shows.

SOMEONE IN APPARENT DISTRESS

That sounds genuinely difficult and it is more than I can help with here. Please speak to a doctor or a licensed mental health professional soon. If you are in crisis, your local emergency service or crisis line is the right call right now. I mean that seriously.

Same funnel. No separate diagnostic. No commercial pathway from altered states content.

Bridge content feeds the existing funnel or it feeds nothing. Building a second commercial pathway for depth-interested readers would be the first concrete step toward the merge this assessment recommends against, because a funnel is a commitment in a way that a post is not.

11.1 The funnel

STAGE	CHANGE	NOTE
LinkedIn content	Add bridge posts at max 5% of output	Bridge posts use the same CTA as every other post
Assessment	No change	The Sapiens OS diagnostic stays exactly as it is. No altered states questions.
DM sequence	Add disqualification screen	New rule set in 11.2
Diagnostic call	Add one qualifying question	What are you actually trying to change, and what have you already tried
90 Day Performance Reset	No change	Product does not change. Ever.
Referral	No change	

11.2 Lead qualification and disqualification

Qualify in

- Founder or senior operator with a live commercial context and a specific performance problem.
- Someone describing capacity erosion in concrete terms: sleep, energy, decision quality, reactivity.
- Someone who arrived through a pillar post or a data post and is asking about their own situation.

Disqualify out

- Anyone whose opening question is about substances. This is the single clearest signal of a mismatch.
- Anyone seeking preparation, integration, or support around a specific planned experience. This is not the O!Sapiens product and cannot be sold as one.
- Anyone describing symptoms that belong to clinical care. Route out immediately and warmly.

- Coaches and practitioners seeking training, methodology, or professional collaboration. Polite, brief, no pipeline.
- Anyone in acute distress. Route to professional support. No commercial conversation of any kind.

11.3 DM rules

9. Never sell in the first DM. Never sell in a DM that follows a bridge post.
10. If a conversation opens on substances, it does not convert. Answer once, honestly and briefly, and let it end. Attempting to redirect it toward coaching is the behaviour that would justify every criticism of the category.
11. Never discuss a person's own use, plans, or history. If they volunteer it, acknowledge without engaging and return to the presenting problem or close the conversation.
12. High intent inquiries get the same answer regardless of who is asking. Seniority does not change the boundary.
13. Anyone routed to clinical care gets a follow up in one message and no further contact.

AUDIENCE	DECISION	REASONING
Founder focused on sustainable performance	ATTRACT	The core buyer. Everything in the strategy protects this segment first.
Founder interested in altered states	TOLERATE	May be a strong client for the wrong reason. Do not court, do not reject. Convert only on the performance problem, never on the interest.
Founder in burnout or existential transition	ATTRACT	The highest value and highest need segment. This is precisely who the four existential pillars are written for. They do not need psychedelics mentioned to feel addressed.
Senior leader seeking deeper self-understanding	ATTRACT	Underserved by both executive coaching and wellness. The soft bridge speaks directly to them without any risk.
Psychonaut, not a premium buyer	REPEL	Consumes attention, generates engagement that looks like progress, and never buys. The vocabulary discipline in section 10.2 is the repellent.
Curious coach or facilitator	EDUCATE	No commercial value, meaningful reputational value. Be generous and brief. They become referrers, not clients.
Clinical or research professional	EDUCATE	High credibility value. Engaging seriously with the evidence is what earns their respect and their public association.
Corporate buyer	ATTRACT	The most risk-sensitive audience and the largest future revenue pool. Every red zone rule exists mainly to protect this relationship.

THE ASYMMETRY THAT DECIDES THIS

The audience the merge would attract is the audience that does not buy premium executive coaching. The audience it would repel is the one that does. Engagement would very likely go up. Revenue would very likely go down. This is the specific failure mode of the psychedelic coaching category and it is visible in how its incumbents monetise: they sell certifications to aspiring practitioners rather than coaching to executives, because that is where the actual demand turned out to be.

Assessed for Option C, the controlled merge, since that is the option being informally advocated. Likelihood and severity would fall substantially under the recommended Option B.

RISK	LIKELIHOOD	SEVERITY	EARLY WARNING SIGNAL
Reputational with corporate buyers	High	High	Corporate inbound stops. Existing corporate contacts go quiet without explanation.
Platform action	Low to moderate	High	A post gets limited reach with no obvious cause. A comment is removed.
Clinical claims exposure	Moderate	High	DMs start asking whether Vlad can help with a diagnosed condition.
Legal, Spain	Low	Severe	Any inquiry that treats Vlad as an access point rather than an educator.
Audience confusion	Very high	Moderate	New followers cannot articulate what Vlad does. Comments split into two unrelated conversations.
Commercial dilution	High	High	Engagement rises while qualified DMs fall. The most dangerous pattern because it looks like success.
Wrong client attraction	Very high	Moderate	Discovery calls that open on substances rather than performance.
Scientific credibility	Moderate	High	Researchers and clinicians stop engaging. Comment quality shifts from peer to fan.
Personal oversharing	Moderate	High	A post feels good to write. That feeling is the signal to not publish it.
App and regulatory contamination	Moderate	Severe	Any investor or app store question about brand association with controlled substances.

13.1 Prevention and mitigation

RISK	PREVENTION	MITIGATION IF IT OCCURS
Corporate reputational	Red zone rules. Never on O!Sapiens assets. Bridge content capped at 5%.	Stop bridge content immediately. Publish three pillar posts. Do not explain or apologise publicly.
Platform	Never name a substance in a headline or first line. Always cite a source. Never anything resembling promotion.	Delete, do not appeal, do not post about it. Reduce frequency for 30 days.
Clinical claims	Standing disclaimer. Never respond to a symptom description with a suggestion.	Correct publicly and immediately. Route the individual to care.
Legal	Never facilitate, refer, advise on access, or discuss legality beyond citing a regulator.	Cease the conversation. Retain a Spanish lawyer before any further public comment.

RISK	PREVENTION	MITIGATION IF IT OCCURS
Audience confusion	One position in the headline. Bridge content always ends on executive consequence.	Return to 100% Executive Healthspan for 60 days.
Commercial dilution	Track qualified DMs, not engagement. Weekly.	This is the stop signal. Revert to Option A.
Wrong clients	Disqualification rules. Vocabulary discipline.	Decline warmly and immediately. Do not attempt conversion.
Scientific credibility	Grade every claim. Lead with limitations. Never overstate.	Publish a correction. Corrections build credibility if they are prompt and unhedged.
Oversharing	Never publish a personal post the same day it is written.	Delete. Do not explain.
Regulatory contamination	Absolute firewall between O!Sapiens assets and any substance reference.	Full audit of all product-layer material before any funding or app submission.

COMPETITOR TYPE	OWNS	VULNERABILITY	VLAD'S POSITION
Psychedelic integration coaches	A defined niche and a passionate audience	Unregulated field. Credibility contested. Monetises via certification more than client work.	Do not enter. Nothing to win here.
Executive coaches	Corporate procurement, perceived legitimacy	Purely psychological. No biology. No data.	Offensive. Reframe the level of analysis.
Healthspan and longevity voices	Scientific credibility, large audiences	Consumer general public framing. No coaching relationship. Not executive specific.	Flanking. Executive application and 1:1 depth.
Biohackers	Content volume, protocol specificity	Self-experimentation aesthetic. Low corporate credibility.	Distance deliberately. Never adopt the vocabulary.
Therapists and clinicians	Clinical authority	Cannot address performance context. Cannot operate commercially in the executive frame.	Complement and refer. Never compete.
Leadership consultants	Board access, organisational scale	Structural focus. Individual physiology is entirely outside their model.	Partner potential. Different unit of analysis.
Meditation and mindfulness teachers	State regulation practice, mass adoption	Weak executive framing. Perceived as HR wellness.	Adjacent. Use the mechanism, not the category.
Retreat brands	Experience delivery	Legal exposure. No follow through. High variance in quality.	No overlap. No association.
Ketamine clinics	A regulated legal pathway	Medical only. No behavioural layer. High relapse without structure.	Referral relationship at most, and only inbound.
Clinical researchers	Evidentiary authority	No commercial or executive channel	Cite them. Engage seriously. Never claim their authority.

14.1 The defensible difference

The only person in this field who can read the evidence like a scientist and the room like an operator.

Vlad's actual moat is not the depth of his knowledge about altered states. Many people have that. It is the combination of twenty years of commercial operating experience with genuine research fluency and a coaching practice. That combination is rare and it is the thing that makes a founder

trust him in a room. Adding a psychedelic association does not strengthen it. It obscures it, because the label is loud enough to drown out everything else in the sentence.

Root claim

Performance is biological first.

Three supporting claims

14. Every decision is made from a state, and the state is set by systems most leaders have never examined. Attention, judgement, and reactivity are not stable traits. They are outputs.
15. Capacity is the limiting factor, not motivation. The reason good advice does not stick is rarely that the person disagreed with it. It is that the system receiving it did not have the headroom to hold a new pattern.
16. Insight is cheap and integration is the work. Realisation arrives from many directions. Almost none of it survives a hard week without a structure carrying it into ordinary days.

Proof layer

- Ten pillar Sapiens OS framework built on over a thousand hours of primary research
- Published personal biological data, tracked over time, including decline
- Peer-reviewed anchors used with correct evidence grading, including negative findings
- Twenty years commercial operating experience across Europe and the Middle East
- The O!Sapiens diagnostic as a structured entry point

Boundary statement

STANDING

I am a coach and educator. I do not diagnose, treat, prescribe, facilitate, or work with substances of any kind. If what you need is clinical care, I will tell you so directly.

Category vocabulary

Executive Healthspan. Performance is biological first. Capacity. State dependent. Regulation. Recovery window. Biological debt. Integration, in the behaviour change sense. Decision clarity. Performance without collapse. The ceiling.

Words to retire

High performers, as an audience label. Longevity operating system. Optimisation. Wellness. Balance. Anything from the banned list: unlock, transform, biohack, journey, plant medicine, best version of yourself, game-changer, proven, cure.

Signature phrases

- Performance is biological first.
- Biology sets the capacity. Behaviour sets the outcome.
- Sleep first, then philosophy.
- Insight is cheap. Integration is the work.
- The system matters more than the insight.
- Behaviour change is a skill, not a willpower problem.
- Healthspan is how long you can keep choosing.

Profile tagline

Performance is biological first.

One line introduction

I work with founders and senior leaders on the biology quietly limiting their capacity to lead, decide, and recover.

Short bio

Vlad Andreev is an Executive Healthspan coach and the founder of O!Sapiens. He works with founders and senior leaders on the biological and behavioural systems that set the ceiling on attention, judgement, and recovery. His Sapiens OS framework covers ten of them, built on over a thousand hours of primary research. Twenty years in commercial roles across Europe and the Middle East. Based in Barcelona.

Long bio

Vlad Andreev is an Executive Healthspan coach and the founder of O!Sapiens. He works with founders and senior leaders at the point where high performance has quietly become a debt, paid in sleep, decision quality, and reactivity.

His starting position is that this is biological before it is psychological. Most executive development assumes a stable physiological baseline and addresses strategy, habits, and mindset. For the leaders who need it most, that assumption has already failed, which is why the standard toolkit stops returning results at exactly the point it is most needed.

The Sapiens OS framework covers ten systems that set capacity. Six are physical: sleep, nutrition, movement, stress load, light exposure, and rest. Four are not: agency, meaning, social connection, and curiosity. He treats the second group as measurable inputs to physiology rather than as soft topics, because that is what the evidence supports.

Before this he spent twenty years in commercial roles across Europe and the Middle East, in negotiation, brand strategy, and business development. Education at UC Berkeley, Yale, and IMD. Certified coach. He came to this work from operating, not from wellness, and he publishes his own biological data, including the parts that are going the wrong way.

He is a coach and educator. He does not diagnose, treat, prescribe, or provide therapy. Based in Barcelona, working in English and Russian.

16.1 Five posts, conservative bridge strategy

POST 1. AGENCY

Most leaders think they have control over their week.

Run the actual test. Look at the last fourteen days and count the hours where you decided what happened, versus the hours where something else decided and you responded well.

For most senior people the second number is much larger than they expect. And the gap matters more than it sounds, because perceived control is not a mood. It is a physiological input.

The research on this is old and consistent. Lower perceived control over demand produces higher baseline cortisol and slower recovery after stress, independent of workload. Same hours, different biology, depending on who is choosing.

Which means two executives can work identical weeks and one of them ages faster.

This is why time management interventions so often fail at senior level. They optimise the schedule without changing who owns it. The nervous system is not tracking hours. It is tracking whether the load is yours.

Pillar 7 of Sapiens OS. Agency is not a leadership virtue. It is a recovery variable.

POST 2. INSIGHT AND BEHAVIOUR

Almost every leader I work with already knows what is wrong.

They can name it in the first fifteen minutes. Usually with precision. Sometimes they have known for years.

So the interesting question is not why they cannot see it. It is why seeing it changed nothing.

The standard answer is discipline, and the standard answer is wrong. Knowing and doing are handled by different systems, and the one that does depends on resources the one that knows does not require. Recognising a pattern is cheap. Interrupting it under load is expensive, and the load is exactly when it matters.

This is why realisations that feel enormous on a Sunday evening are gone by Wednesday. Nothing was wrong with the realisation. There was just no structure underneath it and no capacity available to run it.

Insight is cheap. Integration is the work. And integration is mostly unglamorous: smaller than you want, slower than you want, and starting from a system that can actually hold it.

Which is why I start with biology. Not because meaning matters less. Because a depleted system cannot act on meaning.

POST 3. MEANING

There is a version of burnout that has nothing to do with hours.

The person is sleeping adequately. The workload is manageable by any objective measure. And they are flat in a way that does not respond to rest, which is confusing to everyone including them.

What has usually happened is that the work stopped meaning anything, and nobody counted that as a physiological event.

It is one. The systems that sustain effort over long horizons are not driven by workload. They are driven by whether the effort connects to something the person actually wants. When that connection breaks, motivation does not degrade gracefully. It goes, and the body follows.

I am careful here, because meaning is where this conversation usually turns into something soft and unfalsifiable. So let me keep it narrow: the absence of meaning shows up in measurable ways, in sleep, in recovery, in inflammatory markers, in the same places everything else shows up.

Which makes it a performance variable, not a philosophical one.

Pillar 8. If you are rested and still cannot start, the problem is probably not rest.

POST 4. REST

Time off is not recovery. They are different things and only one of them counts.

Most senior leaders get a reasonable amount of the first and almost none of the second. Two weeks away, phone technically off, and they return with a resting heart rate that has not moved.

The reason is that the nervous system is not tracking location or calendar. It is tracking whether the open loops are closed. Unresolved decisions, unfinished conversations, things that will be waiting.

As long as those are live, the system keeps a background process running and never fully downregulates. You are on a beach. Your physiology is in the meeting.

This is why the leaders who recover well are not the ones who take more time. They are the ones who have some ritual that convinces the system the work is genuinely contained rather than postponed. Written down, handed over, decided, closed.

It sounds trivial. It is the whole mechanism.

Pillar 9. Rest is not the absence of work. It is the presence of closure.

POST 5. CURIOSITY

Curiosity is the first thing stress takes and the last thing anyone thinks to measure.

Under sustained threat, the system narrows. This is adaptive and it is fast. Attention goes to the immediate, the known, the previously successful. The range of options a person can even generate shrinks, and here is the part that makes it dangerous: it does not feel like narrowing from the inside. It feels like focus. It feels like decisiveness.

So the leader who has become least able to consider an unfamiliar option often experiences themselves as unusually clear.

I see this constantly in founders eighteen months into something difficult. They are not out of ideas because the market changed. They are out of ideas because a stressed nervous system stops producing them, and nobody around them can tell the difference between conviction and constriction.

The fix is not a brainstorming session. It is upstream, in the physiology.

Pillar 10. Cognitive flexibility is a biological capacity, not a personality trait. Which means it can be restored.

16.2 Five posts, controlled merge strategy

Publish at most three of these in the 90 day window, and only after all ten pillars are live.

POST 1. THE MICRODOSING EVIDENCE

A founder asked me last month whether microdosing would help his focus.

He expected me to be either enthusiastic or dismissive. The accurate answer is neither, and it is worth stating plainly because almost nobody in this conversation is incentivised to.

The controlled evidence is not encouraging. A preregistered meta-analysis published in Neuroscience and Biobehavioral Reviews in early 2026 pooled fourteen studies and around sixteen hundred participants. No overall cognitive benefit. One domain, cognitive control, was slightly worse than placebo. A phase 2A randomised trial of repeated low-dose LSD for adult ADHD found the placebo group improved marginally more than the treatment group.

The observational studies look considerably better. That gap is usually the tell. When people who chose an intervention report benefit and people who were randomised to it do not, expectancy is doing most of the work.

None of this is an argument about anything larger. Supervised psilocybin for treatment-resistant depression is a genuinely different question with genuinely stronger data behind it.

But the specific thing a high functioning executive is usually asking about, a small regular dose to sharpen output, is the version with the weakest support. That is worth knowing before you build anything on it.

This is education, not medical advice, and not a recommendation of any kind.

POST 2. WHY INSIGHT FADES

Something I have watched happen repeatedly, in contexts that have nothing to do with each other.

A leader has a significant realisation. In therapy. In a crisis. On a long walk after a bad quarter. In stillness, or in whatever form it arrives for them. And it is real. The thing they saw was true and they had not seen it before.

Six weeks later, nothing has changed.

This is so common that it deserves a better explanation than failure of will. The explanation is that insight and behaviour run on different systems, and the second one costs resources the first one does not. Seeing a pattern is cheap. Interrupting it, repeatedly, under pressure, when the old response is faster and more familiar, is expensive. And the moments where it matters are precisely the moments when resources are lowest.

Which means the durability of any realisation depends less on how profound it was and more on the state of the system receiving it.

I think this is the single most underrated thing in the whole field of personal change. Everyone optimises for the moment of clarity. Almost nobody builds the structure that carries it into an ordinary Tuesday.

Insight is cheap. Integration is the work.

POST 3. WHAT THE CLINICAL EVIDENCE ACTUALLY SHOWS

The clinical picture on psychedelics changed materially in the last eighteen months and most commentary has not caught up in either direction.

What is now reasonably strong: supervised psilocybin for treatment-resistant depression. Three positive phase three trials, over a thousand patients, a rolling regulatory submission underway. That is a real evidence base by any standard, and dismissing it as hype is no longer an informed position.

What that evidence does not say: anything about healthy high functioning adults. Anything about performance, creativity, or leadership. Anything about unsupervised use. Anything about low doses. The population studied was people with a specific diagnosis who had failed multiple prior treatments, in a controlled setting with extensive support. Generalising from that to a founder wanting an edge is not a small extrapolation. It is a different question entirely.

Also worth holding: a mixed methods study of over six hundred people reporting extended difficulties found that for roughly a third, problems lasted over a year. Those occurred in both guided and unguided settings.

I am not offering a view on what anyone should do. I am saying that if you are going to have an opinion about this, it is worth having one that matches the actual literature rather than the version that reaches you through founder dinners.

Education, not medical advice. I do not work with substances.

POST 4. STATE DEPENDENT JUDGEMENT

Every decision you make is made from a state.

This sounds obvious and it is treated as though it were trivial, which it is not. The same person, same information, same stakes, produces measurably different judgement depending on physiological conditions they are usually not tracking and often cannot perceive.

Sleep restriction shifts risk appetite toward reward and away from loss avoidance. Sustained cortisol narrows the range of options generated. Glucose depletion after hours of context switching degrades exactly the deliberative processes you rely on for the decisions that carry longest, and those decisions tend to happen late in the day.

The uncomfortable part is that none of this is perceptible from inside. Impaired judgement does not feel impaired. It frequently feels sharper, because the circuits that would normally introduce doubt are the ones running quietest.

So the practical question for a senior leader is not how to make better decisions. It is what state you are typically in when the consequential ones get made, and whether anything in your week is designed to protect that window.

Most calendars are designed to do the opposite.

POST 5. WHAT I DO NOT DO

Worth stating plainly, because I get asked.

I am a coach and an educator. I work on the biological and behavioural systems that set a leader's capacity. Sleep, stress physiology, metabolic stability, recovery, and the four systems people find less comfortable to put in a performance framework: agency, meaning, connection, curiosity.

I do not diagnose. I do not treat. I do not provide therapy. I do not prescribe. I do not work with substances of any kind, I do not facilitate anything, and I do not make referrals in that area.

I read the research in adjacent fields carefully, including the psychedelic literature, because senior people ask me about it and they deserve an accurate answer rather than an enthusiastic or a dismissive one. Reading a field is not practising in it.

And if what someone brings me is clinical, I say so and point them toward a licensed professional. That happens regularly. It is not a failure of the conversation. It is the conversation working.

I think the boundaries matter more than the offer. An unregulated field is exactly where they matter most.

16.3 Five comments

UNDER A POST CELEBRATING PSYCHEDELIC RESEARCH

The phase three data on supervised psilocybin for treatment-resistant depression is genuinely notable and worth taking seriously. The part that tends to get lost is how narrow the studied population is. Diagnosed, treatment-resistant, supervised, supported. Almost none of the enthusiasm I encounter in founder circles is about that population, and the extrapolation from there to healthy high performers is much larger than it looks in a headline.

UNDER A POST ABOUT FOUNDER BURNOUT

The mechanism worth naming is that the capacity to act on any realisation about burnout is itself degraded by burnout. Which is why founders often describe knowing exactly what they need to do for months before doing any of it. That is not avoidance. It is a system without the headroom to run a

new pattern under load. Whatever the entry point, the physiological floor has to come up first or nothing else holds.

UNDER A POST ABOUT EXECUTIVE DECISION MAKING

There is a state variable underneath most of this that rarely gets included. Sleep restriction reliably shifts risk appetite toward reward. Sustained cortisol narrows the set of options a person can generate. And neither is perceptible from inside, because the circuits that would flag the impairment are the ones most affected. The decisions that feel most certain are sometimes made in exactly the state where certainty is least informative.

UNDER A POST MAKING AN OVERSTATED WELLNESS CLAIM

Worth separating the observational and the controlled evidence here, because they point different directions on this specific claim. Where people who selected an intervention report benefit and people randomised to it do not, expectancy is usually doing the work. That does not make the reported experience unreal. It does change what can be built on it.

UNDER A POST ABOUT INTEGRATION OR LASTING CHANGE

Agreed on the substance, and the framing I would add is that integration is a capacity question before it is a technique question. The structures that carry insight into behaviour all cost something to run, and they cost most in exactly the weeks that test them. Which is why change tends to survive in people whose baseline was stabilised first, regardless of where the insight came from.

16.4 Five DM openers

AFTER A SUBSTANTIVE COMMENT ON A PILLAR POST

That was a sharper read than most of what I get on these posts. Curious what prompted it. Is this something you are seeing in your own week or observing across a team?

AFTER SOMEONE ENGAGES WITH A PERSONAL DATA POST

Appreciated you engaging with that one. Most people find published decline data uncomfortable. Are you tracking anything yourself, or is this more of an interest than a practice at this point?

AFTER A FOUNDER DESCRIBES A SPECIFIC PROBLEM IN COMMENTS

You described that with more precision than most people manage. If it is useful, I built a short structured assessment across the ten systems I work on. No pitch attached, it just tends to make the picture clearer. Happy to send it over.

AFTER A LONG CONVERSATION IN COMMENTS

This is running longer than a comment thread deserves. If you want to continue it properly I am glad to. No agenda beyond the conversation itself.

WARM RECONNECTION WITH A LAPSED CONTACT

Been a while. You came to mind reading something about decision fatigue at senior level. How are things holding up on your side?

16.5 Five DM boundaries

SOMEONE ASKS WHERE TO OBTAIN SOMETHING

That is not something I have any involvement in and not a question I will help with, from anyone. I do education and coaching on biological systems. Nothing beyond that.

SOMEONE ASKS VLAD TO GUIDE OR SIT FOR THEM

No. I do not facilitate anything and I am not qualified to. If you are considering something in that area, the honest advice is to take the screening seriously and to involve a clinician, particularly if there is any psychiatric history in your family.

SOMEONE DESCRIBES SYMPTOMS AND ASKS WHETHER VLAD CAN HELP

What you are describing sounds clinical rather than performance-related, and I would be doing you a disservice if I took it on. Please speak to a doctor or licensed psychologist. I mean that as a real recommendation, not a deflection.

A COACH ASKS TO BE TRAINED OR CERTIFIED

I do not run training and am not planning to. Happy to point you at the research I have found most useful if that helps, but there is no programme here.

SOMEONE PUSHES AFTER A BOUNDARY HAS BEEN SET

I understand the frustration and the answer is still no. It is not about you. It is a fixed line I hold with everyone, and it is the reason the work is safe to do at all. I hope you find the right support for it.

17 The 90 Day Experiment

17.1 Profile changes now

- Update the About section to Version 2, the controlled merge. It signals depth without naming anything and carries no risk.
- Update the headline to the second recommended option, adding the line about agency, meaning, and insight rarely surviving Monday.
- Nothing else. No banner change, no title change, no featured section change.

17.2 Profile changes to delay

- Any mention of altered states, psychedelics, or integration in the headline or title. These are the highest visibility, lowest reversibility surfaces on the platform.
- Any change to the O!Sapiens company page. It stays clean permanently, not just during the test.
- Any link between the personal profile and the depth property.

17.3 Tests

TEST	WHAT	WHEN
Content, phase 1	Seven pillar posts. Movement, Stress, Light, Agency, Meaning, Rest, Curiosity. Zero bridge content.	Days 1 to 60
Content, phase 2	Maximum three bridge posts from section 16.2, spaced at least ten days apart.	Days 61 to 90
Comment test	Twenty comments on posts about burnout, decision making, and leadership. Five comments on posts where psychedelics come up, using the section 16.3 templates.	Throughout
DM test	Track opening topic of every inbound DM. Categorise as performance, substance, clinical, or practitioner.	Throughout
About test	Version 2 live from day 1. Watch profile view to connection request conversion.	Day 1

17.4 Metrics

Track weekly. Only the first three matter for the decision.

- Qualified inbound DMs, meaning founders or senior leaders opening on a performance problem. This is the primary metric and the only one that maps to revenue.
- Discovery calls booked, and the opening topic of each.
- Ratio of executive-profile to practitioner-profile new followers.
- Corporate and enterprise inbound, tracked separately. Any decline is a serious signal.

- Engagement volume, tracked but explicitly discounted. Rising engagement alongside falling qualified DMs is the failure pattern.

17.5 Decision thresholds

SIGNAL	READING	ACTION
Qualified DMs up, follower mix unchanged or more executive	Positive	Continue bridge content at 5% cap. Reassess at 180 days.
Qualified DMs flat, engagement up, follower mix shifting to practitioners	Negative, disguised as positive	Stop bridge content. Return to Option A for 60 days.
Any decline in corporate or enterprise inbound	Stop signal	Stop immediately. Do not wait for the 90 days to complete.
More than 15% of inbound DMs open on substances	Wrong audience forming	Stop bridge content. Tighten vocabulary discipline.
Any platform action on any post	Stop signal	Stop for 30 days. Do not appeal, do not discuss publicly.
Depth gap closes after the four existential pillars, before any bridge content	The answer	No bridge content required. Revert to Option A permanently and formalise Option E for the depth work.

THE MOST LIKELY OUTCOME

The last row. On the evidence in this assessment, the probable finding is that the four existential pillars close the depth gap entirely and no bridge content is needed. If that happens, it is not a failed experiment. It is the cheapest possible answer to an expensive question.

DECISION	RECOMMENDATION
Strategic option	F, sequenced, operated as B, with E as the standing architecture. C and D rejected.
Tagline	Performance is biological first.
Headline	Executive Healthspan Coach Performance is biological first Also writing about agency, meaning and why insight rarely survives Monday
About section	Version 2, the controlled merge. Live from day one.
Category	Executive Healthspan primary. Executive Biology as the explanatory phrase. State-aware leadership as the bridge.
Content ratio	50% pillars, 20% recognition and mechanism, 15% evidence, 10% personal data, 5% bridge. Hard cap on the last.
Funnel	Unchanged. One funnel. No separate diagnostic. No commercial pathway from bridge content.

Strongest argument for this strategy

It gets the depth without paying for it. The four existential pillars already sit inside Sapiens OS, they have never been published, and they address exactly the audience the advisors want to reach: founders in transition, leaders questioning the cost of the role, people whose problem is not their sleep. Writing them costs nothing, risks nothing, and is entirely consistent with the position. If the advisors are right that something is missing, this is the cheapest way to find out, and it is reversible at every step.

Strongest argument against it

It is possible that the actual differentiator being left on the table is not depth in general but this depth specifically. There is a version of the future where the field normalises, a Compass approval lands, corporate attitudes shift within eighteen months, and the people who staked a public position early own the category while everyone else is arriving late to a conversation that has already been settled. This assessment recommends waiting, and waiting has a cost that is real and not fully knowable. If psilocybin is approved in a major market and the executive conversation shifts materially, this recommendation should be reopened rather than defended.

The one thing Vlad must not do

DO NOT PUT ANYTHING SUBSTANCE-RELATED IN THE HEADLINE, THE TITLE, OR ON ANY O!SAPIENS ASSET.

Everything else in this document is a tuning decision that can be reversed within a week. This one cannot. The headline is the highest visibility, most indexed, most screenshotted, most permanently cached surface Vlad controls. Once it is there, it is in search results, in AI answers,

in the memory of every person who scanned the profile and moved on, and in the due diligence of every corporate buyer for years afterward.

And the O!Sapiens brand has to stay clean for a reason that has nothing to do with reputation. It is going into a regulated EU product environment. Substance association at the brand layer is a material commercial risk to the app, not just an aesthetic one.

First ten actions

17. Update the About section to Version 2. Today. It is the highest value, lowest risk change available.
18. Update the headline to the recommended option. Do not add anything else to it.
19. Write and publish Pillar 3, Movement, connecting to the VO2 max data already out there.
20. Write and publish Pillars 4 and 6, Stress Regulation and Light and Darkness. Close the physiological half.
21. Write and publish Pillars 7, 8, 9, and 10: Agency, Meaning, Rest, Curiosity. This is the actual test of the hypothesis and it happens before any risk is taken.
22. Set up the tracking spreadsheet for qualified DMs, opening topic, and follower profile mix. Without it the 90 day test produces opinion rather than data.
23. Adopt the content policy in section 10 as a written standing rule. Print it. It only works if it is a rule rather than a judgement call in the moment.
24. Audit every existing O!Sapiens asset, site, deck, diagnostic, and app material, for any substance-adjacent language. Remove it.
25. Formalise the depth property as the standing home for the other lane, with its own audience and no link to O!Sapiens.
26. At day 60, before publishing any bridge content, review the qualified DM data and ask honestly whether the depth gap closed. If it did, stop here.

A

Evidence Appendix

Findings graded by strength. This grading should be reproduced in any public content that references these areas.

Strong evidence

- Supervised psilocybin for treatment-resistant depression. Compass Pathways reported positive results in COMP005 (June 2025) and COMP006 (February 2026), with 26 week durability data in July 2026. Rolling NDA submission underway, final module expected Q4 2026. Serious adverse events were comparable between the 25mg and control arms. Population: diagnosed, treatment-resistant, supervised. Says nothing about healthy adults or performance.
- Microdosing does not outperform placebo for cognition. A preregistered meta-analysis in Neuroscience and Biobehavioral Reviews (January 2026), 14 studies, N=1614, found no overall cognitive benefit and a selective reduction in cognitive control. A JAMA Psychiatry phase 2A RCT of repeated low-dose LSD for adult ADHD found placebo improved slightly more than treatment.
- Expectancy explains much of the reported microdosing benefit. A double-blind placebo-controlled study in Translational Psychiatry found subjective effects and EEG changes without evidence of improved wellbeing, creativity, or cognitive function.

Moderate evidence

- Extended difficulties following psychedelic use are more common than commonly reported. A mixed methods study in PLOS One (N=608) found problems persisted over a year for roughly a third of respondents and over three years for around a sixth, occurring in both guided and unguided settings. Note this is a self-selected sample of people reporting difficulties, so it describes character and duration rather than population prevalence.
- Adverse event reporting in psychedelic trials has been incomplete. Cited work indicates a substantial share of serious and non-serious adverse events went unreported in published findings.

Emerging or contested

- MDMA-assisted therapy for PTSD. FDA rejection in August 2024, application resubmitted August 2026 without a new efficacy trial. Status unresolved.
- 5-MeO-DMT for treatment-resistant depression. Positive phase 2b topline in July 2025, with Spanish trial sites involved. Phase 3 planning underway.

No evidence

- That altered states improve leadership capacity, executive decision quality, or organisational performance. There is no controlled trial of this claim. This is the single most important finding in the assessment, because it is the claim a merged position would implicitly rest on.

Anecdote and observation, clearly labelled as such

- Public founder discourse. Sam Altman has described a psychedelic experience as significantly changing his mindset. Set against this, a named tech CEO was removed by his board after microdosing before an investor meeting, and a prominent founder publicly observed that nearly all the founders he knew who took psychedelic self-discovery trips left their CEO roles within a year. Both directions are anecdote. Neither should be cited as evidence. Both are relevant to reputational modelling.

Regulatory and legal, Spain and EU

- Spain: psilocybin, MDMA, LSD, DMT, and mescaline are listed under the psychotropic control regime. Personal use and possession are tolerated in practice. Supply is a criminal offence under Article 368 of the Penal Code. There is no legal therapy or compassionate access pathway for classic psychedelics. Esketamine has been publicly financed since November 2022 for a narrow treatment-resistant depression subgroup.
- A 2025 Madrid High Court ruling held that ayahuasca as such is not a scheduled substance under Spanish law. This is a classification precedent, not a therapy licence, and should not be cited as one.
- Czech Republic became the first EU state to explicitly regulate medical psilocybin use, effective January 2026, via a strictly clinical route.
- Nothing in this appendix is legal advice. Spanish counsel should be retained before any public position that could be read as facilitation.

Platform

- LinkedIn's Professional Community Policies prohibit promoting or attempting to purchase illegal or dangerous goods and services, and prohibit content that directly contradicts guidance from leading global health organisations and public health authorities. Enforcement ranges from reduced visibility to permanent account restriction.
- LinkedIn is a designated Very Large Online Platform under the EU Digital Services Act, which raises its obligations around public health and illegal content for EU users specifically. Vlad is an EU user.
- Practical reading: citing published research with sources is low risk. Anything that reads as endorsement, facilitation, or personal practice is not.

Competitive

- Bloomberg has characterised psychedelic executive coaching as an offshoot of the unregulated coaching field, which it sized at around five billion dollars. Incumbents in the space monetise substantially through practitioner certification rather than executive coaching, which is a meaningful signal about where demand actually sits.

- Psychedelic integration coaching is explicitly unregulated in most jurisdictions, with certification providers stating openly that there is no official recognition of the occupation.

END OF DOCUMENT

Strategy Board Assessment. O!Sapiens. August 2026. Not legal or medical advice.